

GHRP-2

GH Secretagogue | 5mg & 10mg x 10 Vials

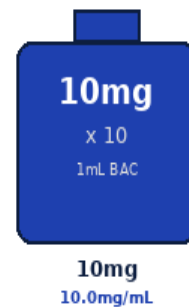
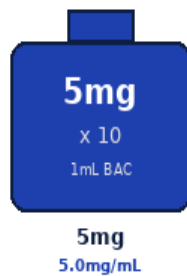
Strong GHS -- Fasted 2-3x Daily | Stack with GHRH for Max GH

Purity: 99.8% | Endotoxin-Free | Research Use Only

GHRP-2 (Growth Hormone Releasing Peptide-2) is a synthetic hexapeptide GH secretagogue and ghrelin receptor agonist. It produces strong GH pulses with a mild cortisol and prolactin elevation (unlike Ipamorelin). Highly effective in the GHRH + GHRP synergy stack. Must be injected fasted.

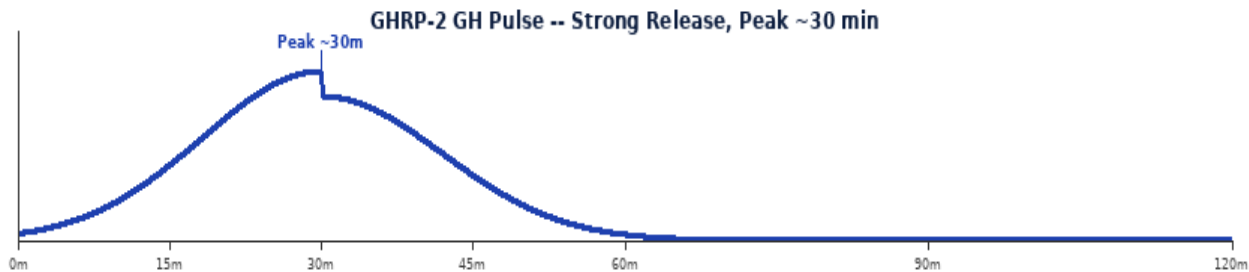
FASTED REQUIRED: Inject at least 1-2 hours after last meal. Insulin blunts GH release. Wait 30 min before eating post-injection.

GHRP-2 Vial Comparison



Parameter	Specification
Class	GH Secretagogue (GHS) -- ghrelin receptor agonist (GHSR-1a)
Vial Sizes	5mg x 10 10mg x 10 Purity 99.8%
Reconstitution	1 mL BAC Water: 5mg->5mg/mL 10mg->10mg/mL
Standard Dose	100-300 mcg per injection -- always fasted
Frequency	2-3x daily fasted morning + pre-bed + optional midday
vs Ipamorelin	Stronger GH pulse; slight cortisol/prolactin spike; mild hunger increase
Cycle	12 weeks on, 4 weeks off
Storage	2-8 deg C Reconstituted: use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
GHSR-1a	Binds ghrelin receptor on pituitary somatotrophs and hypothalamus; activates Gq/IP3/calcium cascade; strong GH granule exocytosis; larger GH pulse than Ipamorelin	Potent GH release; effective muscle building protocol
Cortisol/ Prolactin	Unlike Ipamorelin, GHRP-2 has mild ACTH/cortisol and prolactin co-stimulation; usually minor at standard doses; relevant consideration for longer cycles	Monitor for elevated cortisol/prolactin at high doses
GHRH Synergy	Stacked with CJC-1295 or Sermorelin: GHRH primes pituitary + GHRP-2 triggers powerful release; amplified GH 3-10x vs either alone	Maximum GH when stacked; best body composition results

Benefit	Context
Strong GH Output	GHRP-2 produces larger GH pulses than Ipamorelin, making it preferred for aggressive muscle-building and performance protocols.
GHRH Synergy	Combined with CJC-1295 or Sermorelin, GHRP-2 produces amplified GH pulses via dual-pathway activation.
Fasted Timing	Fasted injection is mandatory. Avoid eating within 30 min post-injection for maximum GH effect.

RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



1mL Insulin Syringe | 29-31G | SubQ

Formula: $\text{Vol(mL)} = \frac{\text{Dose(mcg)}}{\text{Conc(mcg/mL)}}$ | **Units:** $\text{Vol} \times 100$

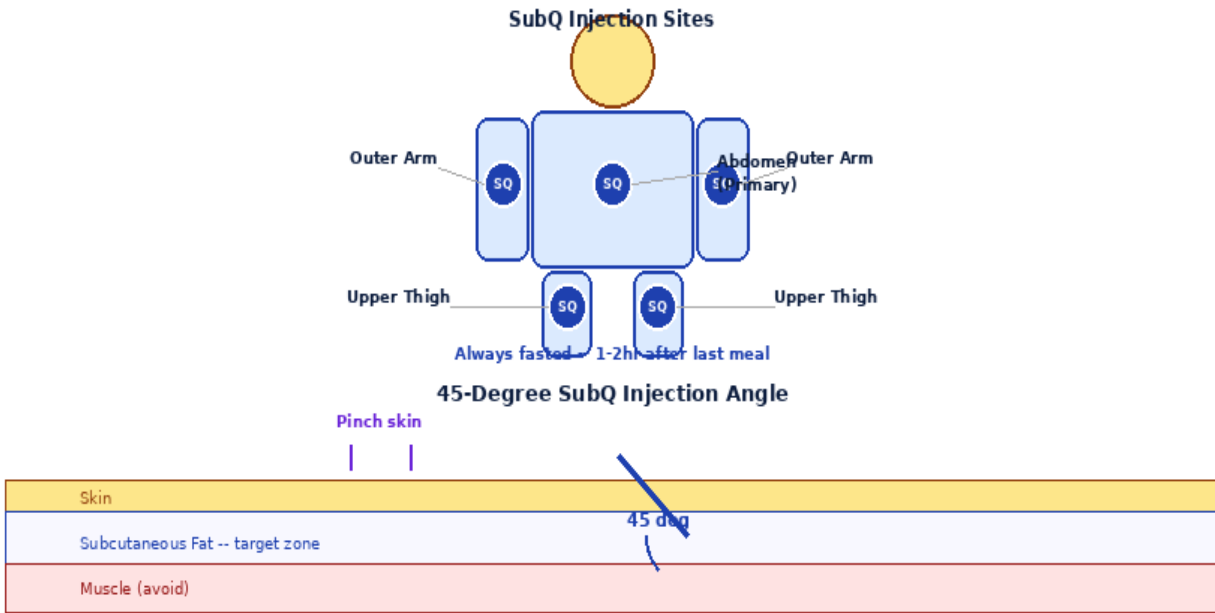
5mg x 10 -- 1mL BAC -> 5.0mg/mL (5,000mcg/mL)

Dose(mcg)	100mcg	150mcg	200mcg	250mcg	300mcg
Vol(mL)	0.0200	0.0300	0.0400	0.0500	0.0600
Units	2.0U	3.0U	4.0U	5.0U	6.0U
Doses/Vial	50	33	25	20	16

10mg x 10 -- 1mL BAC -> 10.0mg/mL (10,000mcg/mL)

Dose(mcg)	100mcg	150mcg	200mcg	250mcg	300mcg
Vol(mL)	0.0100	0.0150	0.0200	0.0250	0.0300
Units	1.0U	1.5U	2.0U	2.5U	3.0U
Doses/Vial	100	66	50	40	33

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Confirm fasted	1-2 hr after last meal. Mandatory.
2	Draw dose	Draw exact volume. If stacking with GHRH, can combine in one syringe.
3	Inject SubQ	Abdomen preferred. Pinch 2-3cm. 45 deg. Rotate sites.
4	Wait 30 min	Before eating. Record site. Sharps container.

Protocol	Dose	Frequency	Duration	Notes
Standard	200 mcg	2-3x daily fasted	12wk on 4wk off	Both injections must be fasted.
+GHRH Stack	200mcg GHRP-2 +CJC-1295	2x daily fasted	12wk on 4wk off	Combine in syringe. 3-10x GH amplification.

Side Effect	Likelihood	Management
Injection site reaction	Common	Rotate sites. Fresh needle.
Mild cortisol/prolactin	Occasional	Monitor at high doses/long cycles. Use Ipamorelin if sensitivity.
Hunger increase	Common	Ghrelin receptor activation. Manage with diet.
Water retention	Occasional	GH-mediated. Reduce dose if significant.

Contraindications: Cancer history, pregnancy, under 18.

1. Always inject fasted (1-2hr after last meal). Insulin blunts GH release.
2. 1 mL BAC Water per vial: 5mg->5mg/mL 10mg->10mg/mL.
3. Stack with CJC-1295 or Sermorelin for 3-10x amplified GH pulse.
4. GHRP-2 causes mild cortisol/prolactin elevation -- switch to Ipamorelin if sensitivity.
5. Inject 2-3x daily: morning fasted + pre-bed are optimal windows.
6. Wait 30 min post-injection before eating. Rotate 8-point abdomen map.
7. Cycle 12 weeks on, 4 weeks off. Store 2-8 deg C, 28 days max.

DISCLAIMER: SaiyanMed research use only. Not for human therapeutic use. Consult a qualified professional. SaiyanMed assumes no liability.

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